

# FOOT SYMPTOMS

Check if problem is in the joint: ask patient to walk. Sit and stand up with arms folded. If unable to do all actions comfortably and problem seems to be specifically in the joint → 62.

## Give urgent attention to the patient with foot symptoms and any of:

- Unable to bear weight following injury → 18.
- Sudden severe foot pain at rest with any of the following in the leg: numbness, weakness, pallor, no pulse: **acute limb ischaemia** likely
- Muscle pain in legs or buttocks on exercise associated with foot pain at rest, ulcer or gangrene on foot: **critical limb ischaemia** likely.

Refer urgently.

## Approach to the patient with foot symptoms not needing urgent attention

If cracks/peeling/scaly lesions between toes or thickened scaly skin on soles/heels/sides of feet, **tinea pedis (athlete's foot)** likely → 70.

### Generalised foot pain

Constant burning pain, pins/needles or numbness of feet worse at night

#### Peripheral neuropathy likely

- Test for HIV → 110 and syphilis. If HIV positive, give routine care → 111. If syphilis positive → 53.
- Exclude diabetes → 17.
- In the past year, has patient: 1) drunk ≥ 4 drinks<sup>1</sup>/session, 2) used illegal drugs or 3) misused prescription or over-the-counter medications? If yes to any → 142. If harmful alcohol use, give **thiamine** 100mg daily.
- Give **paracetamol** 1g 4-6 hourly (up to 4g in 24 hours) when needed and **amitriptyline**<sup>2</sup> 25mg (or 10mg if ≥ 65 years) at night. If needed, increase by 25mg (or 10mg if ≥ 65 years) every 2 weeks, up to 75mg at night.
- If on isoniazid, increase **pyridoxine** to 25mg 8 hourly for 3 weeks, followed by 50mg daily. If pregnant, discuss/refer.
- If one-sided, weakness or severe numbness, refer same week.
- If patient known to have cancer, refer same day.
- If no better with treatment, discuss/refer.

Foot pain with muscle pain in legs or buttocks

**Peripheral vascular disease** likely → 139.

### Localised pain Ensure that shoes fit properly.

Heel pain, worse on starting walking

#### Plantar fasciitis likely

- Advise to: decrease aggravating physical activity (like running). Apply ice. Avoid bare feet - wear shoes with good support or use shoe inserts. Stretch and exercise feet regularly: stretch calf muscle/s on waking and before sleep, roll foot/feet over cold drink bottle.
- If BMI<sup>3</sup> > 25, assess CVD risk → 127.
- Give as needed: **paracetamol** 1g 4-6 hourly (up to 4g in 24 hours) or **ibuprofen** 400mg 8 hourly with food for up to 5 days (avoid ibuprofen if peptic ulcer, asthma, hypertension, heart failure or kidney disease).
- Refer for physiotherapy, if available.

Foot deformity  
Bony lump at base of big toe; may have callus, redness or ulcer

#### Bunion likely

- Give **paracetamol** 1g 4-6 hourly (up to 4g in 24 hours) for up to 5 days.
- If severe pain or ulcer, refer.

## In the patient with diabetes or PVD identify the foot at risk. Review more frequently the patient with diabetes or PVD and any of:

- **Skin:** callus, corns, cracks, wet soft skin between toes → 70, ulcers → 75.
- **Foot deformity:** most commonly bunions (see above). If foot deformity, refer for specialist care.
- **Sensation:** light prick sensation abnormal after 2 attempts.
- **Circulation:** absent or reduced foot pulses.

Health for All

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## Advise patient with diabetes or PVD to care for feet daily to prevent ulcers and amputation

- Inspect and wash feet daily and carefully dry between the toes. Avoid soaking your feet.
- Moisten dry cracked feet daily with **emulsifying ointment (UE)**. Avoid moisturising between toes.
- Tell your health worker at once if you have any cuts, blisters or sores on the feet.
- Avoid walking barefoot or wearing shoes without socks. Change socks/stockings daily. Inspect inside shoes daily.
- Clip nails straight, file sharp edges. Avoid cutting corns or calluses yourself and chemicals/plasters to remove them.
- Avoid testing water temperature with feet or using hot water bottles or heaters near feet.

<sup>1</sup>One drink is 1 tot of spirits, or 1 small glass (125mL) of wine or 1 can/bottle (330mL) of beer. <sup>2</sup>Avoid if on bedaquiline. <sup>3</sup>BMI = weight (kg) ÷ height (m) ÷ height (m).